جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : هناء محمد محمد علي
المحافظة : الفيوم **الحي / المدينة :** يوسف الصديق **العنوان :** يوسف الصديق
رقم البطاقة : 27209072300686 **رقم التليفون :** 01095894397 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 07-09-1972 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنه و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: هناء محمد محمد علي
National ID: 27209072300686

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

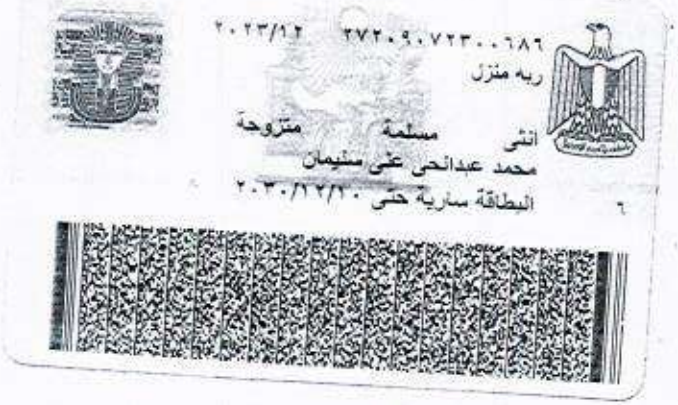
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
هناء
محمد محمد علي
المشرك قبلي
مركز يوسف الصديق - الفيوم
٢٧٢٠٩٠٧٢٣٠٠٦٨٦
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ربه منزل
انثى
مسلمة
متزوجة
محمد عبدالحى على سليمان
البطاقة سارية حتى ٢٠٣٠/١٢/٣٠
٦

DMS

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

07/21/2026

MDT FORM

Name:	Hana Mohamed Mohamed Ali	**MRN:**	2107
Age:	54 Y	**ID:**	27209072300686
Menopausal Sta		**Phone:**	01095894397
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:		**Reason Of MDT:**	

Clinical Data:

BX:

ER: 8/8 **PR:** 7/8 **HER2:** +3 **KI67:**

Sonomammography:

Mets Workup:

History:

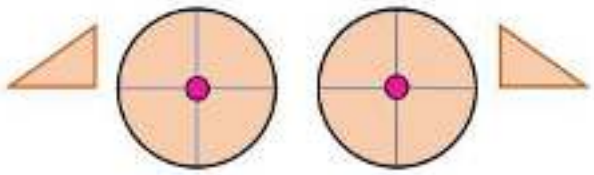
POSTMENOPAUSAL LEFT BREAST CANCER WITH met LN & bone ,ER8/8 PR 7/8 HER2 3 +CT3 N2 M1 REC TAXOLCARBO HERCEPTIN THEN PR , kept on ARIMIDEX +herceptin, last on 22/4, presented with left breat nodule 1.5*1.5cm , SM : showed DP
ct cap 13/5/2026 comparing to 8/2025 >> PD + newly developed bony lesions

Recommendations:

MDT Outcome & Plan

shift to Tykerb - Xeloda
Zometa/3m
Palliative RTH on needed bony sites

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د ندي حسن					

Name: هناء محمد محمد علي	
MR. : 2107	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

Post-menopausal left breast cancer with axillary lymph node involvement and a newly developed left sacral ala lesion (ER8/8, PR7/8, HER2 3+). The patient was started on a taxol-carboplatin-herceptin regimen followed by adjuvant aromatase inhibitor therapy (arimidex) from August 2024, with a new multidisciplinary team (MDT) review for continued HER2-targeted therapy and aromatase inhibition.

In early 2026 the patient's left breast mass was noted on examination, with an ejection fraction of 66 % and a body weight of 55 kg. The patient was receiving HERCEPTIN 330 mg and ARAMUDEX as part of ongoing maintenance therapy.

A pharmacy visit on 1 April 2026 confirmed the administration of HERCEPTIN 330 mg and ARAMUDEX (C23).

On 22 April 2026 the patient reported that the scheduled surgical mastectomy (SM) had not been performed. A CT of the left hip joint was completed but pending report, and a CT pelvis was scheduled externally. Imaging revealed a left breast mass measuring 1.5 × 1.5 cm with axillary lymph nodes 2 × 2 cm. The patient was to receive HERCEPTIN 330 mg plus arimidex (C24). The last ZOMETA dose was in October 2025; a dental examination was planned if a new intervention was considered. An echocardiogram had been performed two months prior, with a new echo requested.

On 6 June 2026 the patient was off treatment since March 2026. The plan was to continue maintenance HERCEPTIN 330 mg plus aromatase inhibitor therapy. An echocardiogram was ordered, and a CT of the chest, abdomen, and pelvis (CT CAP) was scheduled after consultation with Dr. Hanan. The patient was to continue maintenance HERCEPTIN and AI therapy.

On 6 June 2026 the MDT recommended further imaging (CESM, bone scan, echocardiogram) and the initiation of neoadjuvant chemotherapy with Taxol-Carbo for 12 weeks (with Keytruda if approved by MOH), followed by AC plus Keytruda for four cycles. The patient was to undergo left breast conservation surgery (BCS) with sentinel lymph node biopsy if unifocal disease was confirmed. The first week of chemotherapy (W1) consisted of Taxol 75 mg/m² and carboplatin AUC 2 with supportive therapy; laboratory results were pending. Weight was 80 kg, height 170 cm, and Keytruda approval was pending. Weeks 2 and 3 were to include Taxol-Carboplatin (TC) with pharmacy support.

On 1 July 2026 a CT CAP performed on 13 May 2026 showed progression disease and newly developed bony lesions. The MDT discussed switching to Tykerb (neratinib) and Xeloda (capecitabine), with Zometa every three months and palliative radiotherapy to symptomatic bony sites (left hip pain, ECOG 1). Chemo labs, calcium levels, echocardiogram, and dental assessment were scheduled.

On 12 August 2026 the MDT confirmed the shift to Tykerb and Xeloda, with Zometa every three months and palliative radiotherapy as needed. Chemo labs were accepted for Xeloda; Tykerb approval was pending, as was the echocardiogram and dental assessment. The plan was to start Xeloda until

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

Tykerb approval, to discontinue hormone therapy, with ECOG 1 and BSA 1.6 for Xeloda 3 + 3 dosing.

On 7 September 2026 the patient developed grade 2-3 diarrhea after the first cycle of Xeloda. The dose was reduced to 2 × 2 mg and the patient was to be followed up after one week. If no improvement, a DPYD gene test would be performed. Tykerb was to be administered for five cycles (5 × 1) and Xeloda for two cycles of 14 days each (2 × 2 × 14). A new bone scan was scheduled, with a potassium supplement of 3.3 mg. Follow-up was planned after one week, with an ejection fraction of 68 % on 7 September 2026. The patient has completed 1 cycle of Xeloda and 5 cycles of Tykerb to date.

Patient's Name:	**Hanaa Mohammed Mohammed Ali**
Date:	**2026-05-13**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Compared to last study dated 4-8-2025 current study revelas

I- CHEST:

- Mild progressive course as regard the Left axillary lymph node measuring 16 x 15 mm compared to 13 x12 mm in last study.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation.
- Stationary course as regard the Right hepatic lobe segment VI hypodense focal lesion is seen measuring 15 x 14 mm compared to 13 x15 m in last study . No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Scanned skeleton : newly developed

- L1 vertebral body intraosseous lytic lesion seen
- and left ala of sacrum large lytic bone lesion is seen measuring 3.6 x3.3 in axial dimensions
- Left femoral head intra osseous lesion likely bone island

Patient's Name:	**Hanaa Mohammed Mohammed Ali**
Date:	**2026-05-13**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Opinion:

- **Mild Progression Left axillary lymph nodes for sonographic assessment.**
- **Stationary course as regard Right hepatic lobe focal lesion? metastatic.**
- **Newly L1 vertebral body and left ala of sacrum lytic bone lesions? metastatic for further metabolic study.**

Dr.sherif Wagdy
Dr.sherif Wagdy

Patient Name	: الأستاذه / هناء محمد محمد على	Request No.	: 13001320
Patient No.	: 27209072300686	Request Date	: 14/12/2023
Path No.	: SR-1590/23	Verification Date	: 30/12/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 51 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast cancer
Nature of Specimen: Slide revision

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (Allred score 7/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 15% nuclear reaction of tumor cells.

Conclusion:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 15% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	الأستاذه / هناء محمد محمد علي	Request No.	13001320
Patient No.	27209072300686	Request Date	14/12/2023
Path No.	SR-1590/23	Verification Date	30/12/2023
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	51 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar
Dr. Heba Hasan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) ـ الدور الأول ـ مبنى رقم (987) ـ كورنيش النيل ـ مصر القديمة ـ القاهرة ـ مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

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Patient's Name:	HANAA MOHAMMED MOHAMMED
Date:	2026-04-22
Referred by:	Prof.Dr.
Hospital:	Goustave Roussey International Egypt

جوستاف روسي

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: pathologically proven left breast cancer on underwent NACT and on targeted therapy, follow up study.

Findings:

Right breast.

- Extremely dense breast parenchyma that lower the sensitivity for mammography. (ACR D)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Evidence of scattered benign macro-calcifications.
- Ultrasound demonstrated heterogeneous breast parenchyma with mottled appearance yet no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Extremely dense breast parenchyma that lower the sensitivity for mammography. (ACR D)
- Axillary tail irregular equal density mass lesion with coarse heterogenous microcalcifications and indistinct margins.
- LOQ, segmental pleomorphic micro-calcifications measuring 52 x 32 mm seen 15 mm from nipple.
- Scattered benign macro-calcifications.
- Ultrasound demonstrated heterogeneous breast parenchyma with mottled appearance.
- 1 o'clock, posterior third, oval heterogeneous mass lesion with indistict margins, micro-calcifications within measuring 26 x 8 mm.
- AXillary irregular anti-parallel heterogeneous hypoechoic mass lesion measuring 22 x 12 mm ?? primary Vs nodal.
- Normal skin/nipple/areola complex.
- More than three pathological axillary: levels 1, 2, & 3 LNs are noted. No pathological supra-clavicular lymph nodes.

Conclusion:-

Right breast (BIRADS 2): Marked breast adenosis & benign macro-calcifications.
Left breast (BIRADS 6): Progressive course regarding the previously reported malignant process.

Best Regards,

DR. Amal Hegazi, M.Sc.

Patient Name: Hanaa Mohamed Mohamed Ali	Date : 23/08/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 53 Year(s)
Medical ID: 4672/2023	Pat. ID: 45008

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after neoadjuvant chemotherapy of pathologically proved left breast cardinoma.

- Breast density: Extreme dense breast tissue . *Breast density matches ACR D.*
- Area of suspicious punctate calcifications is seen in the left central par of the breast over a distance of 3.3 cm.
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the right breast with benign scattered calcifications ..
- Normal skin and nipple.

By complementary breast ultrasound:

- Complete disappearance of the previously described malignant mass seen in the left breast .
- No other suspicious masses in other quadrants.
- No detected suspicious masses in the right breast .
- Normal skin, subcutaneous fat and nipple areola complex on both sides.
- Few (about 3 in number) suspicious looking levels 1& 2 left axillary lymph nodes, the largest measures (14 x 13 mm) forming mass like lesion in the left axilla .

Conclusion:

- Comparing to the previous radiological data .....findings are suggestive of partial radiological response (PR)(BIRARDs 6 category).
- Benign findings of the right breast (BIRADS 2 category).

Much Obliged

DR.MONA ZAKY.MD

professor-Breast imaging-Mansoura university

Signature Dr. Mona Zaki

23/08/2025 11:58 AM

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Hanaa Mohamed Mohamed Ali	Date : 10/12/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 53 Year(s)
Medical ID: 4672/2023	Pat. ID: 45008

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:

**** Correlation with the previous study dated 4/8/2025**

1. **Chest:**
   - **No appreciable changes** regarding the size & number of multiple left axillary level I and II pathologically enlarged lymph nodes the largest still measures 13 x12
   - Still noted few tiny pulmonary nodules, likely insignificant.
   - Still noted sub centimetric mediastinal lymph nodes, the largest at perivascular space measures 12 x6 mm.
   - No evidence of pleural effusion or masses seen.
   - Borderline cardiac size. No pericardial effusion detected.
   - Normal CT appearance of the thoracic aorta and the great vessels.
2. **Abdomen and pelvis:**
   - Enlarged liver with **still noted** hypodense non enhancing focal lesion seen at segment VI of the right lobe. (It still measures roughly about 13 x 15 mm in its axial diameters).
   - No intra-hepatic or extra-hepatic biliary radicles dilatation.
   - Normal size & parenchymal texture of the spleen with no focal lesions seen.
   - No pathological abdominal or pelvic lymph nodes.
   - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
   - Normal appearance of the pancreas and both supra-renals.
   - Underfilled urinary bladder hindering its proper assessment yet no stones.
   - No abdominal or pelvic masses or collections.
3. **The scanned skeleton:**
   - Left femur neck bone island.
   - Mild forward slippage of L5 over S1 vertebral bodies due to bilateral L5 pars break (Grade-I lytic spondylolisthesis).
   - Bilateral osteolytic lesions of iliac bones at iliac side of SIJ Likely part of sacroiliitis

OPINION:

Status post follow up showing:

- **Overall stationary course of the study.**

Signature Dr . Nermien Ramadan
24/12/2025 1:34 PM

Name : Hanaa Mohammed Mohammed Ali
Branch : Dokki

Date : 20-09-2025 09:12
Medical No. : 206630
Ins.Company : التأمين الصحي

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History:

- 52-year-old lady, with history of left breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired [illegible] post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may suggest for metastatic bone disease could be seen allover the [illegible] skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Hanaa Mohamed Mohamed Ali	Date : 04/01/2024
Age : 51 Year(s)	Patient ID: 45008
Exam : C.T. ABDOMEN &PELIVS WITH CONRAST	Total DLP (mGy.cm):

POST-ENHANCED MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

**** Correlation with the previous study dated 16/12/2023.**

1. Chest:

- Multiple left axillary level I and II pathologically enlarged lymph nodes the largest measures 1.3x2.1 cm.
- Few pleural and fissural based tiny pulmonary nodules, the largest is still seen at the middle part of the right oblique fissure measuring 4 mm.
- Prominent mediastinal lymph nodes seen subcentimetric in their short axis.
- No evidence of pleural effusion or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with a hypodense focal lesion seen at segment VI of the right lobe showing at portal phase discontinuous contrast enhancement seen with no progressive fill. It measures about 16x18 mm in its axial diameters.No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Hanaa Mohamed Mohamed Ali	Date : 10/08/2024
Exam : C.T CHEST WITH CONTRAST	Age : 52 Year(s)
Medical ID: 4672/2023	Pat. ID: 45008

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- Mild forward slippage of L5 over S1 vertebral bodies due to bilateral L5 pars break (Grade-I lytic spondylolisthesis).

OPINION:

Status post follow up showing:

- ***Regressive course regarding the size and number of the left axillary lymph nodes.***
- ***Stationary rest of the study.***

Dr. Mostafa Abdelwahab, MSc

Signature Dr. Mohamed Mohsen
14/08/2024 07:42 م

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Hanaa Mohamed Mohamed Ali	Date : 04/01/2024
Age : 51 Year(s)	Patient ID: 45008
Exam : C.T. ABDOMEN &PELIVS WITH CONRAST	Total DLP (mGy.cm):

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- Mild forward slippage of L5 over S1 vertebral bodies due to bilateral L5 pars break (Grade-I lytic spondylolisthesis).

OPINION:

Status post follow up showing:-

- ***Few pleural and fissural based tiny pulmonary nodules for follow up.***
- ***Right hepatic focal lesion showing contrast pattern in keeping with hemangioma.***

Dr. Alaa R. Hani, M.Sc., FEBR
Dr. Amr Farouk, M.D.

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name : Hanaa Mohamed Mohamed Ali	Date : 18/12/2023
Exam : C/T CHEST	Age : 51 Year(s)
Medical ID: 4672/2023	

OPINION:

- *Left pathological axillary lymph nodes as described.*
- *A small hypodense hepatic focal lesion as described, likely metastatic, for further evaluation by contrast enhanced study.*
- *Few pulmonary micronodules likely benign (inflammatory) in nature for follow up.*
- *No definite osseous metastatic deposits.*
- *L5/S1 Grade-I lytic spondylolisthesis.*

Dr, Hala Sayed, MSc

Dr. Sherif Sameh, M.D.

ظة الفيوم - مديرية التضامن الاجتماعى
عية العلمية لرعاية مرضى الأورام
المشهرة برقم ٣٥٦ لسنة ٢٠٠٠

FOC
مركز أورام الفيوم

التاريخ :- 10/11/2023

م المريض:- هناء محمد محمد

طبيب المعالج:- أ.د/ ـــــــــ

Sono-mamography examination of Bilateral Breast Revealed

ACR C : dense fibro glandular breast tissue.

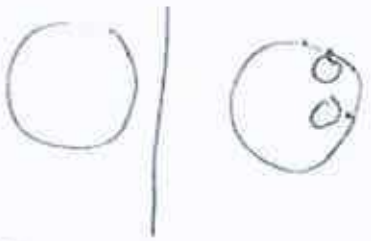
Right breast:-

- Normal fibro-glandular pattern with no evidence of solid masses or cystic lesions could be detected.
- Normal appearance of lactiferous ducts with no evidence of focal lesions within.
- ***Scattered macro- calcifications are noted.***
- Average skin thickness.
- Normal appearance of retro-mammary area and pectoral muscles.
- No axillary /intra-mammary Lymphadenopathy.
- ***Few variable sized axillary lymph nodes are noted. they showed regular outlines and preserved fatty hilum.***

Left breast:-

- ***Multiple irregular shaped highly dense lesion with coarse heterogeneous calcification are seen at the following site:***
  - ***The largest One seen at 4 o'clock measuring (3.1 x 1.2 cm) at maximum transverse diameter with dense calcifications***
  - ***Another one seen at 2 o'clock measuring (2 x 0.9 cm) at maximum transverse diameter .***

Rt Lt



عنوان : إمتداد ش النبوى المهندس - خلف مستشفى الصدر - الفيوم - مصر
تليفون : ٠٨٤٢١٨٤١٧٤ محمول : ٠١٠٦٩٦٩١٠٤٥
البريد الالكترونى : foc600600@gmail.com

جمهورية مصر العربية
بطاقة تحقيق الشخصية



هناء
محمد محمد على
المشرك قبلى
مركز يوسف الصديق - الفيوم

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ربه منزل

انثى مسلمة متزوجة
محمد عبدالحى على سليمان
البطاقة سارية حتى ٢٠٣٠/١٢/١٠

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GUSTAVE ROUSSY INTERNATIONAL

جوستاف روسي انترناشونال

ECHO REPORT

NAME: [illegible] DATE: 7 / 1/2026

ID NUMBER: / SEX: ♀ AGE: 53

EF	68.1.	55-75%	PWT	1.0	0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AO.R	2.7	3-3.7cm
LVESD		2-4cm	LA	3.5	2-4cm
FS		25-45%	RV		0.7-2.3com(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions — oimal

LV Wall Thickness — oimal

LV Diastolic Function — oimal

SWMA — o SWT ↑D

LA and Ao Root Dimensions — oimal

Mitral Valve — oimal

Aortic Valve — oimal

Tricuspid Valve — trivial TR

Pulmonary Valve — oimal

Right Side and Systolic Function — oimal

Intracardiac Masses and Effusion — o

DR.

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بطاقة تحقيق الشخصية



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DR.

جوستاف روسي

إسم المريض ثلاثي: .....
رقم الملف: .....

تقريـــر طبـــي

السيد/.....

تحية طيبة وبعد،،

إيماء للطلب المقدم من ..... بشـان تقريـر طبـي للمـريض: م/ ح.ح.ح.....

بقسم.....رقم طبي .....

نتشرف بأن نوضح فيما يلي تقريرا عن حالته :

تاريخ وساعة الدخول..... .....تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

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تحريرا في

الوقت: ..... **التاريخ :** / /

الطبيب المعالج

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يعتمد ،،